

Statement of the Problem

The crucial procedure known as emergency room (ER) triage establishes the order of importance of patient care according to the severity of problems. Patients frequently feel confused, anxious, and unsatisfied throughout the triage procedure, despite its significance in guaranteeing prompt and effective treatment. Patients' perceptions of triage contacts, waiting decisions, and communication with healthcare workers are less well understood, despite the emphasis on efficiency and clinical correctness. Attempts to enhance patient-centered care in emergency situations may be hampered by this lack of knowledge about patients' actual experiences during triage.

Understanding patient experiences during triage is also crucial because it can show how systemic pressures like congestion and scarce resources affect how fairness and treatment quality are perceived. In-depth examination of these lived experiences offers insightful information on discrepancies between clinician priorities and patient expectations, which may help enhance workflow, communication, and patient-centered emergency department procedures.

Purpose of the Study

This qualitative study aims to investigate patients' actual experiences during the emergency room triage procedure. The goal of the study is to comprehend how patients view triage processes, interactions with triage nurses, and the psychological effects of wait periods and priority. The results will assist in pinpointing areas where patient comprehension, contentment, and general care quality during ER intake can be enhanced.

Additionally, this study looks at how environmental stresses including crowding, noise, and wait times, as well as perceived fairness of priority and information clarity, affect patients' trust in the

healthcare system. Additionally, the study intends to add to the body of knowledge on patient experience in high-acuity care settings and produce useful suggestions for improving patient-centered communication techniques during triage. In the end, the goal is to educate improvements that may be used in clinical practice in addition to describing experiences.

Research Questions

1. How do patients explain their experiences in the emergency department throughout the triage process?
2. How do patients see staff and triage nurses' communication?
3. How do patients feel when they are being triaged and waiting for medical attention?
4. What aspects of the triage process affect patients' comprehension and satisfaction?

Significance of the Study

Because triage is the initial point of clinical contact in the emergency department and establishes the tone for the whole patient experience, this study is important. This research can help develop techniques to enhance communication, transparency, and emotional support during triage by concentrating on patient views. Improving the triage process may boost patient satisfaction with emergency care services, lower anxiety, and build trust.

Additionally, the results of this study should help legislators and healthcare administrators detect gaps between patient expectations and clinical objectives throughout the triage process. The study supports the establishment of training programs for triage nurses that prioritize empathy, communication, and cultural competence and adds to the expanding emphasis

Understanding patient views can help direct measures meant to lower perceived wait times, increase the clarity of triage choices, and provide a more supportive environment, which makes

the study important for quality improvement projects as well. From an academic standpoint, this study contributes to the qualitative literature in emergency care and lays the groundwork for further research examining interventions or contrasting triage experiences in other healthcare environments. on patient-centered care by emphasizing patient voices.

Research Design

In order to document the lived experiences of patients undergoing triage in the emergency department, this study will employ a qualitative phenomenological research approach. Semi-structured interviews with patients who have just finished the triage procedure will be used to gather data.

Purposive sampling will be used to choose participants in order to guarantee diversity in terms of age, gender, and presenting conditions. To find recurrent themes and patterns, interviews will be audio recorded, verbatim transcribed, and subjected to thematic analysis. Peer debriefing, member verification, and keeping an extensive audit trail are all steps to guarantee reliability.

Definition

- Triage: The method of effectively allocating appropriate care by ranking patients according to the severity of their conditions.
- Emergency Room (ER): A medical facility that offers prompt care for serious injuries and diseases.
- Patient Experience: How patients view care, including emotional support, wait times, and communication.

- Phenomenology: A qualitative research methodology that investigates people's actual experiences.
- Thematic analysis: A technique for finding themes and patterns in qualitative data.

Limitations

A tiny, non-random sample that might not accurately represent all patient populations could limit the study. Participants' accounts of their triage experiences may be impacted by recall bias.

Furthermore, results could not apply to all emergency rooms because they are context-specific.

The use of self-reported data, which could be impacted by participants' feelings, recollections, or individual expectations of care, is another drawback. Participants may be more inclined to volunteer if they had exceptionally good or bad experiences, which could lead to response bias.

Additionally, because the study is restricted to a single location, it might not accurately represent differences in triage procedures among hospitals, geographical areas, or healthcare systems.

The number of interviews done may be limited by time and resource constraints, which could limit the breadth and depth of viewpoints recorded. Furthermore, participant replies may be influenced by the researcher's presence during interviews (a phenomenon known as social desirability bias), as people may alter their responses to conform to perceived expectations.

Lastly, because this is a qualitative study, the conclusions are interpretive and depend on the researcher's methodology. Subjectivity in data interpretation cannot be completely eliminated, despite measures to guarantee reliability.

Delimitations

Only adult patients (18 years of age and up) who have received triage in a single urban emergency room during the previous three months will be included in this study. Pediatric

patients, medical professionals, and anyone who are unable to engage in interviews because of cognitive or language problems will not be included. Furthermore, the study will just look at the triage portion of the ER experience; it won't look at the diagnostic, treatment, or discharge procedures that follow.

To ensure that participants can give thoughtful and informed answers, the study will only include patients who are medically stable at the time of recruitment. Due to a lack of resources for translation and interpretation services, the study will only include participants who speak English.

Additionally, since the investigation's goal is to provide a detailed knowledge of experiences within a particular setting rather than making broad generalizations, it will not compare various triage systems or hospitals.

Assumptions

Participants in the study are expected to give candid and thoughtful descriptions of their triage experiences. Additionally, it is assumed that these experiences can offer significant insights into more general patient perception patterns. Additionally, it makes the assumption that qualitative approaches are suitable for examining the intricacy of the triage procedure.

Additionally, it is believed that participants are able to reliably recall their experiences throughout the allotted three months and that their memories are detailed enough to allow for thematic analysis. The study makes the assumption that the triage procedure is an important and memorable part of the ER visit, making it appropriate for further investigation.

Furthermore, the researcher believes that the semi-structured interview method enables rich, thorough data gathering and that participants interpret questions consistently. In order to ensure

that findings accurately reflect participant viewpoints, it is also assumed that the researcher will maintain objectivity and reduce personal bias throughout data collection and analysis. Lastly, notwithstanding participant differences, it is anticipated that the themes found in the data will represent common elements of the triage experience.

References

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